

Three common cataract patterns

The opacity's position explains the pattern of blur and glare.



Cortical cataract: radial spoke-like lens opacities.

Compare the patterns

Pattern	High-yield association
Nuclear	Central yellow-brown sclerosis; increasing index gives a myopic shift and possible second sight.
Cortical	Radial spokes from hydration changes; glare is common.
Posterior subcapsular	Granular posterior central opacity; steroid exposure, glare and near difficulty.

Treatment depends on useful vision

Cataract surgery is considered for cataract-related functional limitation, not acuity alone.

From complaint to treatment

Assess acuity, refraction, glare and the effect on daily tasks.



Confirm that the cataract is the important cause of the limitation.



Discuss extraction and intraocular lens implantation when the expected benefit justifies surgery.

Recognise recurrent blur after surgery

A cataract has no definitive medical treatment; spectacles may help the refractive component while the lens remains opaque. Months or years after successful surgery, posterior capsule opacification may cause painless recurrent blur behind a clear implant. Visually significant capsule opacity is treated with Nd:YAG posterior capsulotomy. This is distinct from removing the original cataract.